

Request for a change in the way I work

Clause 10.3.2, Certified Agreement (No. 12) 2025 · s 27, Industrial Relations Act 2016 (Qld)

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

Employee reference MSH-INJ-5795

To: Injury Management, Human Resources, Logan Bayside Health Network

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Cc: LBH.HRTeam1@health.qld.gov.au

Mr Scott Hughes, Director, Corporate Services

Ms Chloe Taylor, Manager, Switchboard Services

Date: [SEND DATE]

1. WHAT THIS DOCUMENT IS

This is a request under clause 10.3.2 of the certified agreement for a change in the way I work.

I make it as a separate document, rather than within my response to the Request for Medical Information of 31 July 2026, so that there is no question about what it is or when it was received.

Clause 10.3.2 provides that, in accordance with the Industrial Relations Act 2016, an employee "may ask the employer for a change in the way the employee works, including the employee's ordinary hours of work". Section 27(1)(c) of that Act provides that the request may be for "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness or injury".

Clause 10.3.3 requires that the request be in writing, state the change in sufficient detail to allow a decision to be made, and state the reasons. Parts 2 and 3 below do each of those things.

I make the request under two separate limbs of section 27(1), and I ask that they be considered separately.

(a) **ORDINARY HOURS** — Part 2.1. This is made under section 27(1)(a), which concerns "the employee's ordinary hours of work", and under clause 10.3.2, which is in the same terms. That limb stands on its own. It does not depend on illness or injury, and neither section 27, nor section 28, nor clause 10.3 makes medical information a precondition to making such a request or to deciding one. My reasons for it are matters of rostering history and are set out at Part 3.

(b) **THE WAY I WORK** — Parts 2.2 to 2.5. These are made under section 27(1)(c), which concerns "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness or injury".

I separate them because a decision on one is not a decision on the other, and because the question of my ordinary hours does not require, and should not await, information sought for a different purpose. If the Health Service is not in a position to decide (b) at present, I ask that it decide (a), and say so.

2. THE CHANGE I AM ASKING FOR

2.1 MY ORDINARY HOURS

That my rostered pattern continue at the level at which it has in fact operated over the twelve months preceding the Employee Capabilities Checklist of 3 July 2026.

The Employee Capabilities Checklist describes that level, at Additional Comments 3, as a "reduced pattern of approximately six shifts per fortnight", and records that it "is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation". The first page records the arrangement as a "continuation of existing arrangement".

I am not asking for any reduction from the pattern I have been working. I am asking that the existing pattern continue. It was set by the Health Service, worked by me, and sustained across twelve months. No concern was raised with me at any time that it was insufficient, and no request was made that I work more.

I record for completeness that my substantive appointment is full-time at 76 hours per fortnight, and that the pattern described above is less than that. It is, however, the pattern the Health Service in fact rostered and accepted throughout the twelve months in question, and the Employee Capabilities Checklist describes it as a "continuation of existing arrangement". Clause 10.3.2 expressly contemplates a request concerning "the employee's ordinary hours of work",

which is why I have made the request under that clause rather than assuming the position.

If the Health Service takes the view that the requirements of the role cannot be met on that pattern, I ask that it identify what changed, and when, given that the pattern operated for twelve months without any concern being raised with me.

The Health Service holds the roster and payroll records from which the precise figure can be established. I ask that it be established from those records, and that the figure so established be the one used.

I also record that a temporary reduction of hours was raised with me by the Health Service itself, as a form of support, in the course of a discussion recorded in writing on 30 June 2025. I did not take it up at the time. I mention it only to show that an adjustment of this kind has been contemplated within Switchboard Services before, and is not a novel proposal.

2.2 THE WAY I WORK — ATTENDANCE CONFIRMATION

- (a) A telephone call one hour before each rostered shift, to confirm that I am attending.
- (b) If that call is not answered, or attendance is not confirmed, a second call.
- (c) If attendance is still not confirmed after the second call, the on-call operator is contacted and cover is arranged for the shift.

The measure is contingent, not standing. The on-call is engaged only if the two calls do not produce a confirmation, so it carries no cost on any shift I attend. On the shifts I do attend, which is most of them, the arrangement costs one telephone call.

An on-call arrangement already exists in the unit. On 15 April 2024 the team was advised that after-hours on call had been added, with the days highlighted on the roster and a contact process for urgent matters. What I am asking for is the application of a mechanism Switchboard Services already operates, not the creation of a new one.

To be clear about what this is and is not: it is a confirmation of attendance, of the kind any roster requires, made early enough to be useful. It is not an assessment of my condition, and I am not asking that colleagues on shift observe, assess or report on my mental state. That would place a burden on staff who are not qualified to carry it, would require my health information to be known to co-workers, and would itself be a workplace stressor. No colleague needs to know anything about my health for (a) to (c) to work. They need only know whether the shift is confirmed.

2.3 THE WAY I WORK — IN-SHIFT ESCALATION

A defined escalation pathway which I may initiate during a shift if my condition is acutely exacerbated — including who assumes the board and how the shift is closed out — so that the response is planned rather than improvised.

2.4 THE WAY I WORK — COMPLAINT HANDLING

Continuation of the existing arrangement recorded in the Health Service's own correspondence: immediate escalation of complaints received by Switchboard staff to the Client Liaison Officer and/or the Manager, Switchboard Services. This is the Health Service's established process and it requires no change to anything.

2.5 THE WAY I WORK — CORRESPONDENCE ROUTING

That injury management correspondence concerning me be directed through the Injury Management team, Human Resources. This is what the Employee Capabilities Checklist itself recommends at Additional Comments 4(c): "return-to-work coordination and correspondence through the Injury Management team, Human Resources."

2.6 REVIEW

That the arrangement be reviewed at six weeks, or earlier if either party considers it necessary.

I am willing to discuss any of the above, and any alternative the Health Service or my treating practitioners consider more appropriate. If the Health Service would prefer a graduated commencement, I am willing to return on a graduated basis by agreement.

3. THE REASONS FOR THE CHANGE

3.1 The pattern has been sustained out of my own leave, and that is what has run out.

That roster was the arrangement the Health Service designed. Where my condition was exacerbated and I could not attend a rostered shift, I managed that myself, by taking sick leave or annual leave. No further adjustment was sought from the Health Service and none was provided. The variability was absorbed by me, out of my own entitlements.

That is why, on 3 July 2026, I wrote that my paid sick leave was exhausted. It was exhausted in the course of managing, without assistance, a condition arising in the workplace.

3.2 The change replaces that with a process.

The measures at 2.2 and 2.3 are directed precisely at that. They do not change the roster. They replace an arrangement under which exacerbations are absorbed by my personal leave balances with one under which they are managed by a defined process. That is what a control is, and it is what has been absent.

3.3 Distress peaks as a shift approaches, and the measures are directed at that.

Distress rises as a rostered shift approaches, and there have been occasions where I have called in at short notice as a result. What makes that worse — for the service and for me — is that a late call leaves the board short and the preceding shift unable to leave. The purpose of the sequence at 2.2 is that where I am not going to attend, that is established about an hour ahead rather than at or after start time.

3.4 It is directed at the only matter ever raised with me.

The only matter that has been raised with me in respect of my conduct is attendance and lateness. No concern about the quality of my work has ever been put to me, and the correspondence of 31 July 2026 states that the Health Service is not aware of any performance or conduct matters having been raised for appropriate management. The measures at 2.2 are directed at the one matter that has been raised, and are the measures most likely to prevent its recurrence. Had they been in place, they would have addressed a substantial part of the attendance history recorded in the correspondence of 8 September 2025. I raise that not to reopen it, but because it identifies the control that was missing.

3.5 The applicable policy contemplates exactly this.

HR Policy G03 provides at clause 1.1 that temporary adjustments exist so that an employee may "continue work during their recovery", and at clause 1 that managers are responsible for ensuring that work environments and work "do not unreasonably prevent employment for people". The adjustment contemplated by the policy is the thing that allows work to continue while recovery occurs.

4. THE DECISION, AND WHO SHOULD MAKE IT

Clause 10.3.4 provides that the Health Service may grant this request, grant it in part or subject to conditions, or refuse it. Clause 10.3.5 provides that it may grant it in part or subject to conditions, or refuse it, "only on reasonable grounds".

I ask that the decision be made by an officer outside Corporate Services.

I do not ask that Corporate Services be removed from the management of my employment, and I make no allegation against any individual. Part of what this request concerns is the reporting arrangements within Corporate Services, and the officers who would otherwise decide it sit within those arrangements. Clause 1.2 of the Code of Conduct for the Queensland Public Service provides that a conflict of interest is not wrongdoing in itself, but that failing to disclose and manage one is likely to be wrongdoing. Section 89 of the Public Sector Act 2022 is to the same effect.

The separation I propose is the ordinary one: the clinical material held by Injury Management on a medical-in-confidence basis; the decision made outside Corporate Services; implementation — rostering, and putting in place whatever is decided — retained within Corporate Services. The officer steps out of the decision, not out of the workplace. This requires no restructuring and no finding to be made about anyone.

I ask that the officer who makes the decision be identified to me, together with the source of the delegation relied on.

5. THE FRAMEWORK THE DECISION SITS IN

I set this out for one reason only. Clause 10.3.5 permits the Health Service to grant the request in part or subject to conditions, or refuse it, "only on reasonable grounds". Grounds are reasonable against the whole of the law that applies, not against the agreement alone. If the request is refused in whole or in part, I would like to be able to understand the reasons given under clause 10.3.6 by reference to the same framework the Health Service is applying.

5.1 THE OBLIGATION

The Anti-Discrimination Act 1991 provides at section 15(1) that a person must not discriminate "in any variation of the terms of work", or "by treating a worker unfavourably in any way in connection with work". Section 15(2) provides that "dismissing includes ending the particular work of a person by forced retirement, failure to provide work or otherwise".

Section 11 provides that indirect discrimination happens if a person imposes a term with which a person with an attribute does not or is not able to comply, and which is not reasonable. Impairment is a protected attribute under section 7.

5.2 THE EXEMPTIONS, AND WHAT THEY REQUIRE

Sections 35 and 36 provide that it is not unlawful to discriminate on the basis of impairment where the person would require special services or facilities and supplying them would impose unjustifiable hardship, or where the circumstances of the impairment would impose unjustifiable hardship.

Those provisions are exemptions, and each of them turns on unjustifiable hardship. Section 5 provides that whether something would impose unjustifiable hardship depends on all the relevant circumstances of the case, including the nature of the services or facilities, their cost and the number of people who would benefit or be disadvantaged, the financial circumstances of the person, the disruption that supplying them might cause, and the nature of any benefit or detriment to all people concerned.

HR Policy G03 puts the same test in the Health Service's own words, at clause 2: unjustifiable hardship is "tested against the whole organisation, not a division or unit within the organisation", and "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable".

I would add that the Health Service is a public entity for the purposes of the Human Rights Act 2019, and section 58 of that Act requires a public entity to act and make decisions compatibly with human rights and to give proper consideration to relevant human rights in doing so.

5.3 WHY I RAISE IT, AND WHAT I AM NOT DOING

I have made no complaint under the Anti-Discrimination Act, and this letter is not one. I make no allegation against anyone and I ask for no finding to be made about anyone.

I raise it because the measures at Part 2 are modest ones, and I would like the reasons for any refusal to engage with that. On the shifts I attend, the arrangement at 2.2 costs one telephone call. The roster at 2.1 is the roster the Health Service already sets. The complaint-handling arrangement at 2.4 is the Health Service's own established process. The correspondence routing at 2.5 is what the Employee Capabilities Checklist itself recommends.

If the Health Service considers that any of the measures would impose unjustifiable hardship, I ask only that it identify which measure, and what the hardship is. That is the same thing clause 10.3.6 requires, expressed in the language of the Act.

6. THE TIME FOR THE DECISION

Clause 10.3.6 provides:

"The employer must give the employee written notice about its decision within 21 days after receiving the request. If the employer decides to grant the request in part or subject to conditions or to refuse the request, the written notice about the decision must state the reasons for the decision, outlining the reasonable grounds for granting the request in part or subject to conditions or for the refusal."

Section 28(3) of the Industrial Relations Act 2016 is in the same terms. Section 28(4) provides that a notice granting the request in part or subject to conditions, or refusing it, must state the written reasons outlining the reasonable grounds, and must state that the Commission has jurisdiction to hear and decide a dispute over the request under chapter 6. Section 29 provides that if the written notice is not given within 21 days after the request is received, the employer is taken to have decided to refuse the request.

Twenty-one days from the date of this letter is [21DAYDATE].

I record these provisions because a date has not been available in this matter at any stage. I have asked several times to be told when I would know whether I am returning to work, and I have not been given a date. The certified agreement provides one, and I would much prefer that the request be dealt with by agreement well before it.

7. WHAT THIS REQUEST DOES NOT DO

For completeness:

- (a) This request is separate from my response of the same date to the Request for Medical Information. The matters raised there are not made under clause 10.3 and are not answered by a decision under it.

- (b) This request is separate from any dispute notified under clause 1.11 of the certified agreement.
- (c) Nothing in this request is a concession that I am unfit to attend work. I remain willing and available to work, and have been since 3 July 2026. Identifying hazards and determining control measures is a function of the Health Service under Part 3.1 of the Work Health and Safety Regulation 2011, and nothing here relieves the Health Service of that function or is offered as a substitute for the workplace assessment I have asked for.
- (d) I make this request without waiver of any entitlement, and it is not to be treated as a request for leave of any kind.
- (e) For the avoidance of doubt, the request at Part 2.1 concerning my ordinary hours is not made conditional on the provision of any medical information, and I ask that it not be decided on that footing. If the Health Service considers it cannot decide Part 2.1 without medical information, I ask that it identify the provision that makes such information a precondition to a decision under clause 10.3.4.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

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